

Hospice Care Plan

Eleanor “Ellie” Voss · Age 84

PREFERRED NAME	Ellie	LOCATION	Front bedroom, west side
PRIMARY CAREGIVER	Daniel Voss (son)	LAST UPDATED	July 10, 2026 · 4:32 PM

Document version: v4 · Reviewed by Dr. Sam Okafor, Hospice Physician

CODE STATUS

DNR (Do Not Resuscitate) — on file with hospice.

Signed 12 Mar 2026 · Authorized by Dr. Okafor

Allergies

Penicillin

Latex

Medications

Morphine (oral solution)

PRN

5 mg · Every 4 hours as needed for pain

Give under the tongue or between cheek and gum — do not have her swallow it whole. Wait at least 4 hours between doses, and log the time given so the next caregiver can track spacing.

Lorazepam

SCHEDULED

0.5 mg · Twice daily, morning and evening

Give with a small amount of food if she seems unsettled on an empty stomach. Don't combine with any other sedating medication without checking with the nurse first.

Senna-S

SCHEDULED

2 tablets · Nightly

Give at bedtime with a full glass of water, and encourage fluids through the day — it works better when she's not dehydrated.

Dosing changes are authorized by Dr. Okafor. Nurses may request a change through Bedside.

Care Preferences

Prefers to be called "Ellie", not "Eleanor"

She may not respond to "Eleanor" at all, even from someone who knows her well — it's simply not a name she answers to anymore.

If a visitor uses it, a gentle correction ("she goes by Ellie") is welcome and won't offend anyone.

Enjoys soft classical music in the afternoon

It's one of the few things that reliably keeps her settled during her quieter, dozing stretch of the day.

A phone or small speaker at low volume near her armchair works well.

Watch for: Switching to anything louder or more upbeat — it can unsettle her instead.

Does not want overhead lights on after 7pm — lamp only

Overhead light strains her eyes.

Get the lamp going before the room starts to feel dim, rather than waiting until she's already asked for it.

Watch for: Overhead light switched on in the evening — it can trigger her anxious "turn" around dusk.

Daily Care Routines

Repositioning every 2 hours to prevent pressure sores

Turn her fully onto her side, supporting her back and knees with pillows.

Check her heels, hips, and tailbone for redness.

Log the time on her chart.

Watch for: A red area that doesn't fade within about 30 minutes of the pressure being relieved — flag it to the nurse before her next visit. Caught early, it's far easier to treat than a developed pressure sore.

Oral care after each meal

Use a soft-bristle brush or moistened swab.

Check for dryness, sores, or bleeding gums while you're in there.

Watch for: White patches on the tongue or inside the cheeks — could be thrush. Mention it to the nurse rather than treating it yourself.

Family prefers a text update after every shift, even if uneventful

A short text to Daniel — mood, intake, anything notable — keeps the family reassured without needing a phone call.

"Quiet shift, ate half her lunch, no concerns" is exactly the kind of message they want, even on an ordinary day.

What Settles Her

- Soft classical music
 - Hand lotion & massage
- Dimmed lights / lamp only
 - Repositioned for comfort

Care Team & Contacts

NAME	ROLE	PHONE	AVAILABILITY
Priya Anand	Primary Hospice Nurse	(555) 044-5566	On-call 24/7
Grace Lin	Volunteer Coordinator	(555) 033-7788	Weekdays 9am–5pm
Dr. Sam Okafor	Hospice Physician	(555) 099-1122	Via nurse line
Daniel Voss	Son · Primary caregiver	(555) 011-2233	Text daytime, call evening
Marisol Voss	Daughter-in-law	(555) 011-9988	—

In an Emergency

If Ellie is in distress — labored breathing, unrelieved pain, or a fall — call immediately:

Daniel Voss (son) — (555) 011-2233
Hospice Nurse Line — (555) 044-5566

Hospice Physician — Dr. Sam Okafor

Date reviewed